



KEMH

MENTAL HEALTH

EMERGENCY FILE

Updated: February 2023
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Owner: Women's Health, Genetics and Mental Health

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Purpose of File

The purpose of this file is to:

- Provide information on what to do in a mental health emergency.
- Assist staff in managing mental health patients within King Edward Memorial Hospital.

This file is to be read in conjunction with WHGMH Guideline – “Mental Health Emergency, Management of” - Go to WNHS Intranet, click ‘Policies’ tab, enter search for Mental Health Emergency.

Hard copies of the "KEMH Mental Health Emergency File" (pink folders) are located in:

1. Psychological Medicine – Nurses’ Triage Office, Agnes Walsh House, Level 1, Room 115
2. Emergency Centre – Doctors’ and Nurses’ Workstation
3. Hospital Clinical Managers Office – B Block, Level 4, Room 405A
4. Sexual Assault Resource Centre – Doctors’ office
5. Mother and Baby Unit – Front Nurses’ Station, 11 Loretto Street, Subiaco
6. Maternal Fetal Assessment Unit – Desk next to printer
7. Labour/Birth Suite – Front desk
8. Electronic Copy available

There is one extra hard copy kept at:

Women’s Health, Genetics and Mental Health, 15 Loretto Street, Subiaco – Medical Co-Director’s office.

What to do in a Mental Health Emergency

Remain calm.

Your first priority is safety to yourself, to others (including baby/babies if present), and then to the patient.

Call a [Code Black](#) (Dial 55) if the psychiatric emergency involves a weapon, aggression, suicide, self-harm attempt, or if in doubt.

Inform staff in your area of the situation, including your Nurse/Midwifery Manager. Ensure one staff member stays with patient, **if safe to do so**, until assistance arrives.

If your area has personal duress alarms, ensure staff member is provided with one otherwise utilize Red Emergency Call Bell attached to the wall or, if fitted, under the reception area desk.

Contact Psychological Medicine on 81521

Speak to the Triage Officer (Mon- Fri 0800hrs to 1700hrs). After hours and on weekends, contact Switch (Dial 91) and they will connect you to the on-call Psychiatry Registrar, who will inform the on-call psychiatrist as required.

The attending Psychiatry Registrar or Psychiatrist will complete a mental health and risk assessment of the patient and provide instructions on a treatment plan.

A mental health emergency is a high priority and every effort will be made to provide a doctor as soon as possible.

The psychiatric treatment may include:

- Medications
- Completing a Referral for Examination ([Form 1A](#)) under the Mental Health Act 2014 (MHA 2014)
- Completing a Transport order ([Form 4A](#)) and transfer to an authorised hospital ([Form 4C](#)) or Mother and Baby Unit (Also see North Metropolitan Health Service (NMHS) Mental Health Transport Hub)
- 1:1 mental health nurse special
- Lock down of the area/ward
- Engaging a partner/support person to reassure and help calm the person

Important Phone Numbers

SECURITY Speed dial Mobile Pager	81408 56 0414 930 174 3103
PSYCHOLOGICAL MEDICINE DEPARTMENT	81521
PSYCH MED TRIAGE NURSE PAGER	1635/1639
HOSPITAL CLINICAL MANAGER (After hours) HOSPITAL CLINICAL MANAGER (After hours) Mobile Speed dial to mobile Pager (carried 24hr / 7 days)	1566 0414 930 196 *41416 3419
If a bed is required: Step 1: Request needs to go to patient's local mental health service, see pages 9 -11 . Step 2: If unsuccessful, ring Statewide Assertive Bedflow Manager, 0414 680 097. Step 3: If still unsuccessful, On-Call consultant to escalate to Executive Director, if required.	
WA POLICE	131 444
AMBULANCE	131 233
EMERGENCY	000
Crisis Care Country Toll Free	9223 1111 1800 199 008
MHERL - Mental Health Emergency Response 24 hrs a day / 7 days a week service (Community clinics open till 9pm for after-hours emergencies)	1300 555 788
INTERPRETER SERVICE - TIS On site : On phone :	9225 7700 KEMH C123457 KEMH C123596

Additional Important KEMH Phone Numbers

EMERGENCY CENTRE	81433	Reception Triage
	81431	
MOTHER BABY UNIT	81799	Reception
	81790	Nurse's station
	81771	Nurse's station
Sexual Assault Resource Centre (SARC)	81828	
	1800 199 888	
SOCIAL WORK DEPT	82777	
After Hours call Switch	91 or 82222	
EAST WING CLINIC	81376	Reception
	81377	Back desk
	82100	Booking
AGNES WALSH HOUSE ENQ	81410	
PATIENT ENQ	81869	
AGNES WALSH PSYCHOLOGICAL MEDICINE	81521	

Additional External Numbers

Office of the Chief Psychiatrist www.chiefpsychiatrist.wa.gov.au 8:30am to 4:30pm	6553 0000
Mental Health Commission	6553 0600
TAXI Black and White Swan	13 10 08 13 13 30

Transporting Mental Health Patients

The [NMHS Mental Health Transport Hub](#) provides a number of resources and guidelines to support the safe transport of mental health patients. (Go to WNHS intranet, NMHS home page, click on Directory tab, then A-Z Directory, scroll down to Mental Health Transport)

Resources include:

- Mental Health Transport Flow Chart
- Mental Health Transport Risk Rating Form and Matrix
- Transport Order – Form 4A

And guidelines for

- Safe transport of people with a mental health condition
- Safe transport of individuals with a mental health condition Transport Orders (Form 4A)
- Safe transport of individuals with a mental health condition Apprehension and Return Orders (Form 7D)

If the patient is determined low risk by the psychiatrist then consider transport by private vehicle or taxi with the assistance of their carer/personal support person or volunteer (see guideline).

If assistance is needed from either a Transport Officer or Police Officer to take a referred person to a place for examination by a Psychiatrist or from Mother Baby Unit to another authorized hospital due to a person's condition, and there being no other safe means of transport, consider least restrictive process for transporting a referred person (see guideline).

For maternity patients, obstetric consultation with psychiatry is required.

Public Hospital Contact List - North

Patient Flow Coordinators – North:

Jackie 0406 402 829
John 0415 257 281
Nikki 0404 863 114

<u>Graylands Hospital Campus</u> Triage: Ph: 6159 6407/ 6499 Email: Triage.GH@health.wa.gov.au	<u>Joondalup Health Campus</u> Switchboard: 9400 9400 Bed Manager: Ph: 9400 9038 Fax: 9400 9069 PLN: Ph: 9400 9400 ext 5014 MHOA: via switch: 9400 9400
<u>Sir Charles Gairdner Hospital (SCGH)</u> Switchboard: 6457 3333 ANATOMICAL PATHOLOGY Ph: 6457 2727 Fax: 6457 2620 MHU Bed Manager: Page via MHU switch: 6383 1000 MHOA: Staff Station: 6457 2881 ED PLN: Ph: 6457 6727	<u>St John of God Midland Public Hospital</u> Switchboard: 9462 4000 Bed Manager: Ph: (7am - 10pm): 9462 4996 Ph: (10pm – 7am): 9462 5140 ED PLN: Ph: 9462 4692 Fax: 9462 4185
<u>Selby (Older Adult)</u> Ph: 9382 0800	

Public Hospital Contact List - South

Patient Flow Coordinators – South:

Ian 0406 580 783 (Tues – Sat)
Annika 0404 821 386 (Sun – Thur)

<u>Fremantle Hospital</u> Switchboard: 9431 3333 Fax: 9431 2921 CL: Ph: 9431 3675 Page: #3455 ATT Triage: Ph: 9431 3555 Fax: 9431 3479 Bed Manager (A/hrs): Ph: 0404 890 174 Ph: 9431 3481 Page: #3481	<u>Fiona Stanley Hospital</u> Switchboard: 6152 2222 ED PLN: Ph: 6152 1584 Fax: 6152 7804 CL: Adult Ph: 6152 1586/1587/1581 CL: Perinatal Ph: 6152 1582 MBU: Ph: 6152 7866/7794 Fax: 6152 4867
<u>Peel Hospital</u> Switchboard: 9531 8000 ED PLN: Ph: 9531 8011 Page: #131	<u>Rockingham Hospital</u> Switchboard: 9599 4000 ED PLN: Ph: 9599 4729/4738 Page: #210

Public Hospital Contact List - East

Patient Flow Coordinator – East:

Chris/Vikki 0404 871 033

<u>Armadale Hospital</u> Switchboard: 9391 2000 Bed Manager: Office hours: via switch 9391 2000 A/hours: 0408 771 039 ED PLN: Ph: 9391 2182 Page: #913	<u>Bentley Hospital</u> Switchboard: 9416 3666 Fax: 9416 3875 Bed Manager: Office hours: 0423 799 691 A/hours: 0424 757 186 W8: 9416 3886
<u>Royal Perth Hospital</u> Switchboard: 9224 2244 ED PLN: Ph: 9224 8571 or 9224 3774 Ph: 0404 894 094 Fax: 9224 3636/2212 CL: 0404 894 097	

Public Hospital Contact List – Statewide

Patient Flow Teleconference: 6212 0688

Nurse Director - Mental Health Patient Flow:

Kieran 0414 680 097

Child and Adolescent Mental Health Service (CAMHS) – Perth Children’s Hospital (PCH)

Bed Flow: Ph: 0466 419 342 Fax: 6456 2303

Email: CAMHSIPUBedRequests@health.wa.gov.au

Private Psychiatric Hospitals Contact List

ABBOTSFORD PRIVATE HOSPITAL (Formerly Niola Hospital)

61-67 Cambridge Street, West Leederville	Tel: 9381 1833
	Fax: 9381 7681
Consultant rooms (Kimberley Street)	Tel: 9381 5755
	Fax: 9381 5442
	9200 5832

HOLLYWOOD HOSPITAL

Monash Avenue, Nedlands	Switch: 9346 6000
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THE HOLLYWOOD CLINIC

Reception:	Tel: 9346 6801
Nurses station	Tel: 9346 6806
Nurses station:	Tel: 9346 6822
	Fax: 9346 6829

MARIAN CENTRE

Private Psychiatric Hospital	
187 Cambridge Street, Wembley	Tel: 9380 4999
	Fax: 9388 3179

PERTH CLINIC

29 Havelock Street, West Perth	Tel: 9481 4888
	Fax: 9481 4454

Emergency Psychiatric Medication Kit

Emergency Psychiatric Medication Kits are available in the following locations:

1. Ward 3
2. Ward 5
3. Ward 6
4. Labour and Birth Suite
5. Emergency Centre
6. Adult Special Care Unit
7. Mother Baby Unit
8. Maternal Fetal Assessment Unit
9. East Wing Clinic (resus trolley)
10. Centenary Clinic (resus trolley)

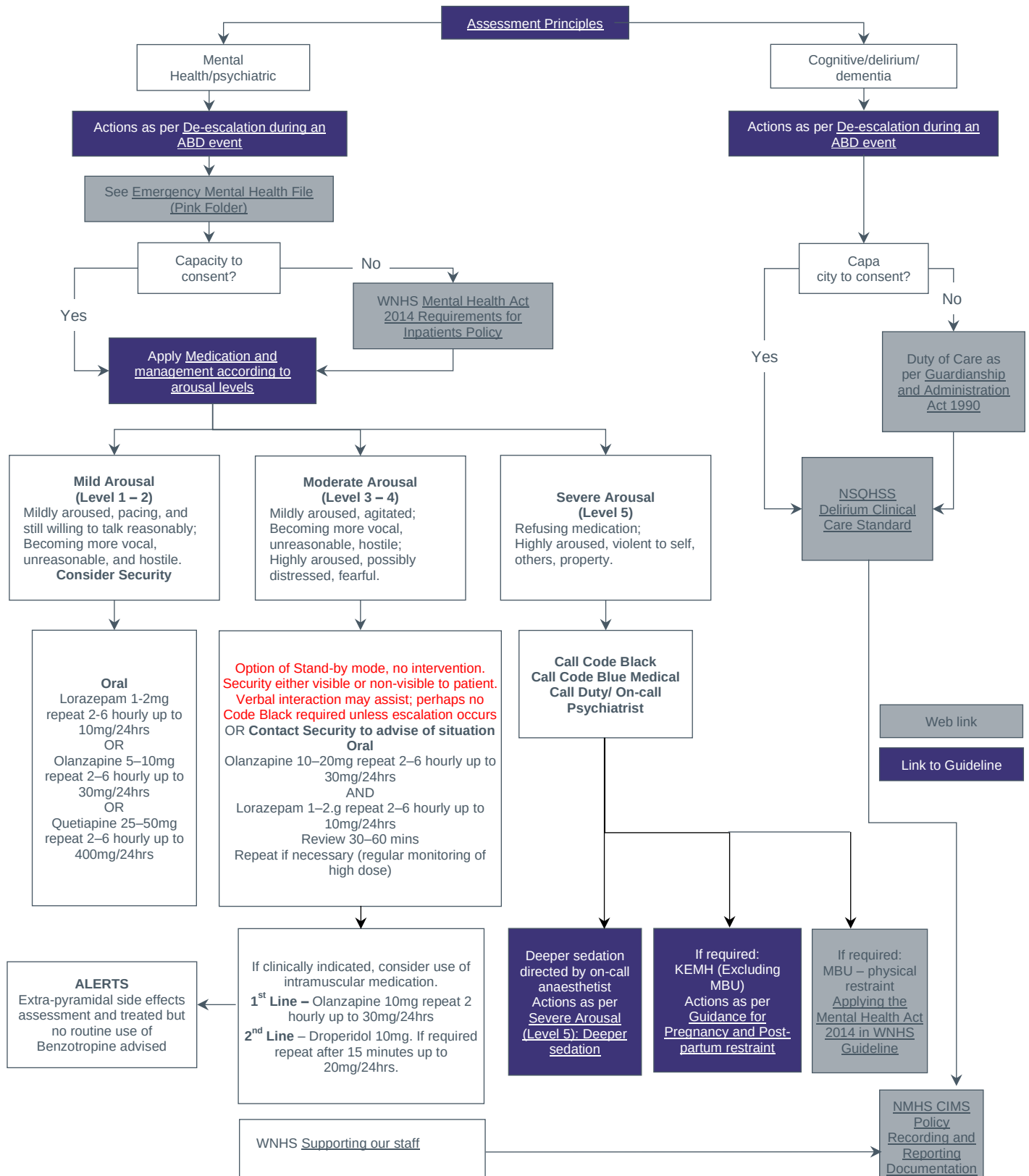
For further information on medication dosage and arousal level, please see the WHGMH Guideline – Mental Health Emergency, Management of. Go to WNHS Intranet, click 'Policies' tab, enter search for Mental Health Emergency.

Current contents
4 x Benzatropine 2mg tabs
5 x Benzatropine 2mg/2ml vials
10 x Clonazepam 500 microgram tabs
2 x Clonazepam 1mg/mL amps
10 x Droperidol 2.5mg/1mL amps
10 x Lorazepam 1mg tabs
5 x Midazolam 5mg/5mL amps
2 x Olanzapine 10mg vials (plus 4 X water for injection 10mL amps)
10 x Olanzapine 5mg wafers
10 x Quetiapine 25mg tabs



Acute Health Related Behavioural Disturbance (ABD) - Inpatients

Flowchart 1 – Acute Health Related Behavioural Disturbance (ABD) – Inpatients



Important Inter/Intranet Links

1. [Clinicians' Practice Guide to the Mental Health Act 2014](#)
2. [ELearning for the Mental Health Act 2014](#)
3. [Mental Health Emergency Response Line](#)
4. [Mental Health Commission](#)
5. [Mental Health Act 2014 Forms – writable and printable \(Office of Chief Psychiatrist\)](#)
6. [Sexual Assault Resource Centre](#)

Physical Restraint

Irrespective of whether the patient has been placed under the Mental Health Act 2014, Duty of Care provisions allow the decision to apply restraint (if not placing the staff member's safety at risk) if the patient attempts to seriously harm themselves (i.e. suicide attempt).

Also see [WHGMH Guideline – Mental Health Emergency, Management of](#) which contains principles of physical restraint in pregnancy and post-partum, principles underlying de-escalation techniques, and acute health-related behavioural disturbance flow charts..

Also see WHGMH Guideline – [Applying the Mental Health Act 2014 in WNHS](#).

If the patient is absconding, note the patient description (i.e. clothing, physical appearance) to report to Police, if needed.

For further information see [Clinicians' Practice Guide to the Mental Health Act 2014](#) – Standard 6: Seclusion and Bodily Restraint Reduction, page 210 – 212.

Mental Health Act 2014 Requirements

See also WHGMH Guideline – [Applying the Mental Health Act 2014 in WNHS](#).

Emergency Psychiatric Treatment

Salient features are as follows:

- At any time during the referral process, the referred person may require treatment such as medication to manage their distress, symptoms or behaviour. Referred persons can be offered treatment and if they provide informed consent to the treatment then it is not deemed to be Emergency Psychiatric Treatment (EPT).
- If a referred person refuses treatment or is unable to consent because of their mental state and they meet the criteria for EPT then they can be provided with treatment without their consent (s. 203).
- The criteria for EPT is that the **treatment is needed to either save the person's life or prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person** (s. 202(1)). EPT cannot be given just because the clinician feels it is appropriate to commence treatment before the person is made an involuntary patient.
- Referred persons are deemed to be voluntary patients and therefore can refuse treatment, so EPT can only be given if the criteria are met.

See also [WHGMH Guideline – Mental Health Emergency, Management of](#).

In circumstances where such provisions apply, such as where security officers and staff hold/restrain the patient whilst the patient is given intra-muscular medication, staff have protection under the Mental Health Act 2014 and are not liable to prosecution after the event.

Note: EPT provides more specific statutory protection for staff than a generic duty of care. It is particularly important to consider the use of EPT rather than duty of care if EPT is provided within an authorised hospital.

In a non-authorised hospital the treatment (usually antipsychotic medication) should then be followed by a Form 9A (Emergency Psychiatric Treatment).

Please note that the word "chemical restraint" is not appropriate, the correct term to use is "emergency psychiatric treatment."

For further information on 'Emergency treatment for referred persons' see page 81 of the [Clinicians' Practice Guide to the Mental Health Act 2014](#):

Detention

Under the Mental Health Act 2014, there are provisions for detention to enable the person to be taken to an authorised hospital or other place, and Emergency Psychiatric Treatment for referred persons.

Therefore, if the decision is made to refer the patient for an assessment by a psychiatrist under the Mental Health Act 2014 (i.e. a Form 1a has been completed), the provisions apply.

Following is a summary taken from the Mental Health Act 2014 Clinicians' Practice Guide.

The most relevant sections are as follows:

Detention of referred patients:

- A medical practitioner or authorised mental health practitioner (AMHP), but not necessarily the medical practitioner or AMHP who made the referral, may make an order (Form 3A) authorising the referred person's detention for up to 24 hours from the time the referral is made, to enable the person to be taken to the authorised hospital or other place for examination.
- This does not mean it should be routine to make a detention order at the same time as a referral order. The elements of referral and detention are separate. Some referred persons will not require a detaining order because they are not trying to leave the place and do not object to being taken to the place of examination. Other referred persons might make it clear of their intention to leave, not cooperate with the transportation, or are so unwell that their behaviour is too unpredictable.
- A detention order can only be made following a referral order.
- The detention order authorises staff to detain the person and under a duty of care the use of reasonable force by staff or others such as security staff. However even when a Detention Order is active, if a staff member is put at risk of physical harm when attempting to detain and decides instead to allow the person to leave, then that may be considered justified as the personal health and safety of staff is of paramount concern. Phone Police or MHERL.
- At times, in order to detain a person, the person may be restrained or placed in a locked room. This is not 'bodily restraint' or 'seclusion' as defined in the Act unless it occurs within an authorised hospital. However, while exercising a duty of care, it is depriving a person of their liberty and can be distressing and should only be used when it is safe to do so and there are no other options to control a situation which may result in harm to the person or others. Using duty of care needs to be justified. The 'doctrine of necessity' will be a defence if there is any civil action.
- For further information on 'Detention to enable a person to be taken to an authorised hospital or other place' - see page 77 [Clinicians' Practice Guide to the Mental Health Act 2014](#):

Transport

For information on patient transfer see WHGMH Guideline [Applying the Mental Health Act 2014 in WNHS](#)

Also see North Metropolitan Health Service – [Mental Health Transport hub for transport](#) flowchart, risk rating form and matrix etc.

For further information on ‘Transport orders’ see page 83 and ‘Transfer between authorized hospitals’ see page 96 of the [Clinicians’ Practice Guide to the Mental Health Act 2014](#):

Search and Seizure

Under the Mental Health Act 2014 the search and seizure provisions apply to all patients, whether voluntary or involuntary.

Salient features relevant to WNHS are as follows:

- All hospitals or mental health services should have policies and procedures regarding search and seizure which sit alongside these legal requirements.
- Search and seizure powers should be used with discretion and never as a routine procedure. Referring a person to be examined by a psychiatrist or making them an involuntary detained patient is not the same as arresting a person or placing a person in custody. In most cases even with people who are significantly mentally unwell there would be no need to conduct a search or seize an article.

The sort of things which could prompt a search are:

- The previous history of the patient
- Being provided with information by a family member or carer
- The patient acting in a suspicious way that they may be secreting an article that might compromise the health and safety of themselves or another person

For example, if the patient has a history of taking overdoses of medication and they are assessed again because they appear distressed and possibly suicidal, then a search for any medication on them would be appropriate.

The police or authorised person may use reasonable force when conducting a search (s. 172(2)). They also have a power to ask another person to assist in the search and that person may also use reasonable force.

For example, if the nurse is authorised then he or she may direct a security officer to assist in the search. However, it is the police or authorised person who is in charge and the person assisting must obey any lawful and reasonable directions from the person authorised (s. 173).

Search and seizure powers apply to voluntary and involuntary patients admitted to a hospital, referred persons whether detained at an authorised hospital or other place, or in fact any other person who presents at a mental health service which can include places such as an ED or a general ward (to the extent that the ward is providing mental health treatment to a patient). There are specific times when the powers are particularly relevant such as at time of admission, returning from leave or when providing treatment (s. 162).

While the search and seizure powers apply to any admitted or detained patient, the approach in relation to voluntary patients who can be searched without their consent needs to be circumspect. A detained referred person can be searched without their specific consent. These issues can be complex and greatly depend on the relationship between the patient and staff. However, the safety of the patient and others is paramount in deciding how these situations should be managed.

For information on Search and Seizure see WHGMH Guideline [Applying the Mental Health Act 2014 in WNHS](#)

For further information on 'Search and Seizure' see page 102 of the [Clinicians' Practice Guide to the Mental Health Act 2014](#):

Notifications

If MHA 2014 Forms are completed on PSOLIS, notifications are sent automatically. The back of each Form contains the list of notifications to relevant parties.

For information on Notifications see WHGMH Guideline [Applying the Mental Health Act 2014 in WNHS](#)

For further information please see the following pages within the [Clinicians' Practice Guide to the Mental Health Act 2014](#):

- Notification of certain events (p.53 – 56)
- Receiving and review of notifiable incidents (p.179)
- Recognition of the rights of carers and families (p.48 - 52)
- Chief Mental Health Advocate (p.145)
- Mental Health Tribunal (p.152)

See also:

Chief Psychiatrist website:

[Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist \(2018\)](#)

[Reporting Checklist \(2018\) Table 1 - Overview of How to Report to the Chief Psychiatrist](#)

[Reporting Notifiable Incidents – Public Mental Health Services](#)

Other websites:

[Mental Health Advocacy Service](#)

[Mental Health Tribunal](#)

[Mental Health Commission - List of Notifiable Events](#)

How to Print Mental Health Act 2014 Forms

Mental Health Act 2014 Forms must be completed via PSOLIS in the first instance. If access to PSOLIS is not available, then a paper copy is to be completed but must be transcribed into PSOLIS at a later date. **See back of file for paper copies.**

The Mental Health Act 2014 Forms can be printed off from the website of the Chief Psychiatrist (along with other related forms) and the Mental Health Unit. These documents are **not** password protected.

Mental Health Act 2014 Forms:

[Office of the Chief Psychiatrist:](#)

This link will bring you to the MHA 2014 Forms page of the Office of the Chief Psychiatrist. It includes both writable and printable forms with instructions.

Office of the Chief Psychiatrist contact details:

- Reception@ocp.wa.gov.au (for general enquiries)
- AMHP@ocp.wa.gov.au (for all matters relating to Authorised Mental Health Practitioners)
- Monitoring@ocp.wa.gov.au (for Monitoring and reporting queries which relate to monitoring)
- MHA2014@ocp.wa.gov.au (feedback path regarding identified difficulties or uncertainties relating to practical translation of the MHA.)

Dr Nathan Gibson
Chief Psychiatrist

1 Nash Street
PERTH WA 6000

GPO Box 45
PERTH BUSINESS HUB WA 6849

Main Reception: (08) 6553 0000
Clinician's Helpdesk: (08) 6553 0016
Fax: (08) 6553 0099

Email: Reception@ocp.wa.gov.au

If form is required to be forwarded to the Mental Health Tribunal - Fax to 9226 2668.



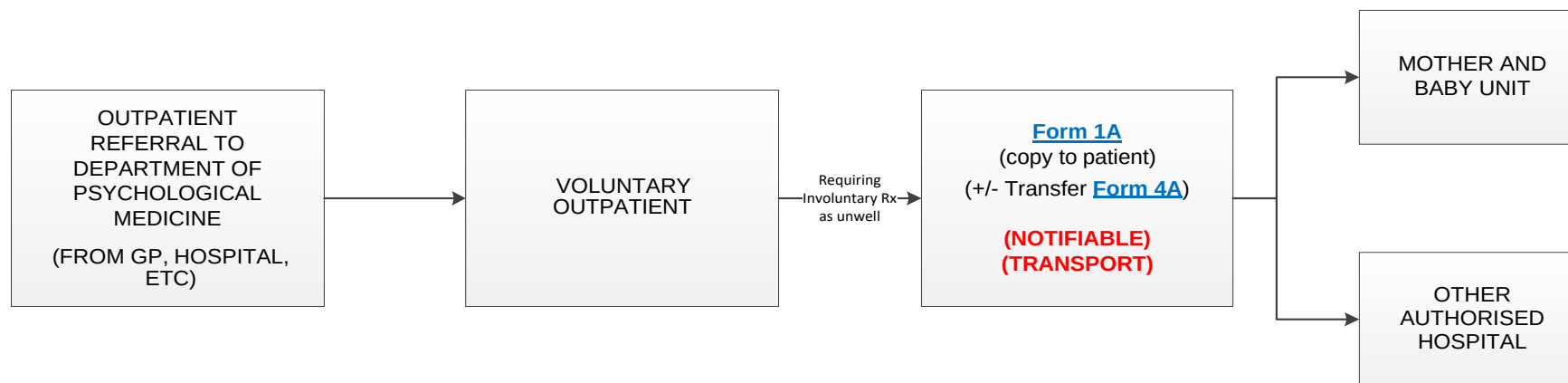
FLOWCHART 1

OUTPATIENT REFERRAL TO DEPARTMENT OF PSYCHOLOGICAL MEDICINE

When to use

For outpatient who has been referred to Psychological Medicine, and who has become so unwell as to be regarded as requiring treatment (or at least consideration of treatment) under the Mental Health Act 2014.

Red Font = Notifiable event



Updated 10 February 2020



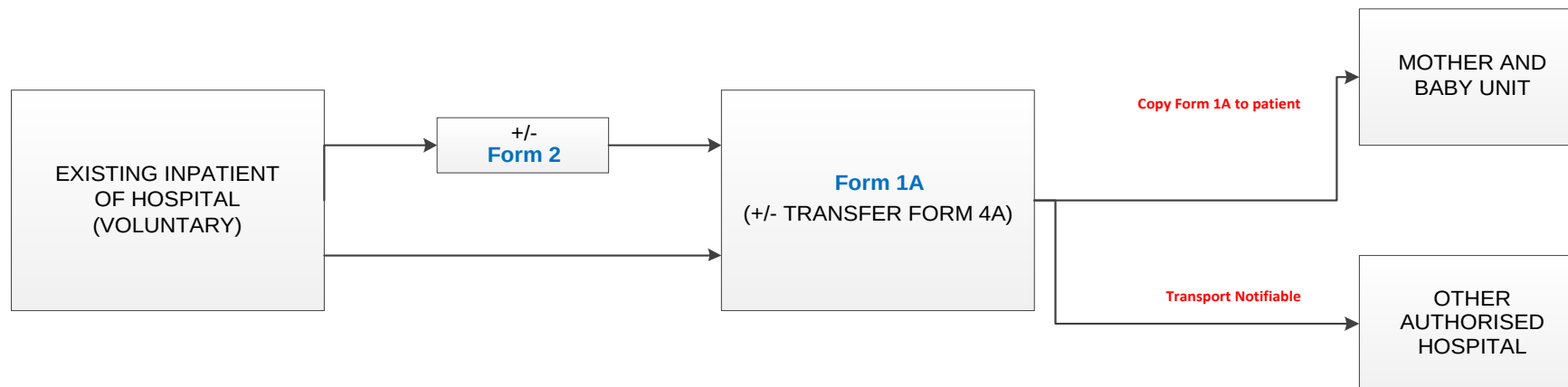
FLOWCHART 2

EXISTING INPATIENT OF KING EDWARD MEMORIAL HOSPITAL

When to use

If an existing inpatient of KEMH becomes so unwell that they require involuntary treatment (or at least consideration of treatment) under the Mental Health Act 2014

Red Font = Notifiable event





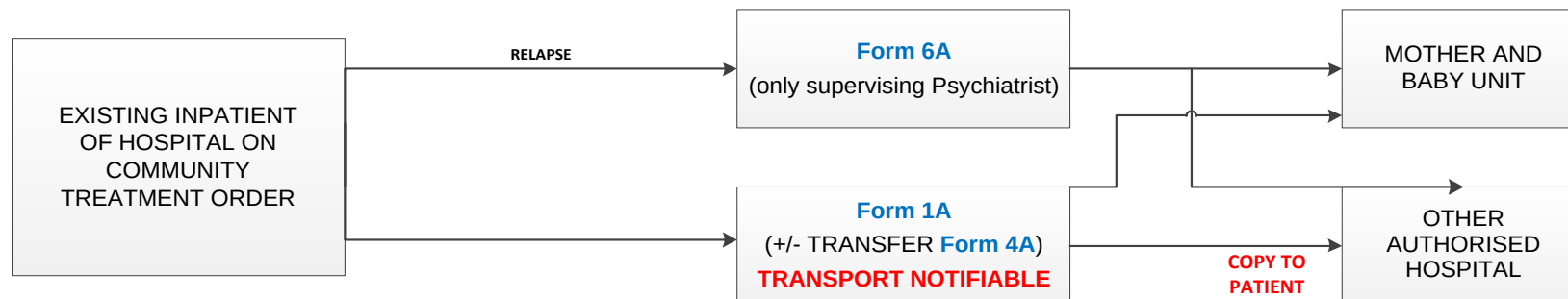
FLOWCHART 3

EXISTING INPATIENT OF KING EDWARD MEMORIAL HOSPITAL ON A COMMUNITY TREATMENT ORDER

When to use

If an existing inpatient of KEMH is currently on a community treatment order (CTO) and becomes so unwell as to warrant revoking of the CTO and requires involuntary inpatient treatment order (ITO).

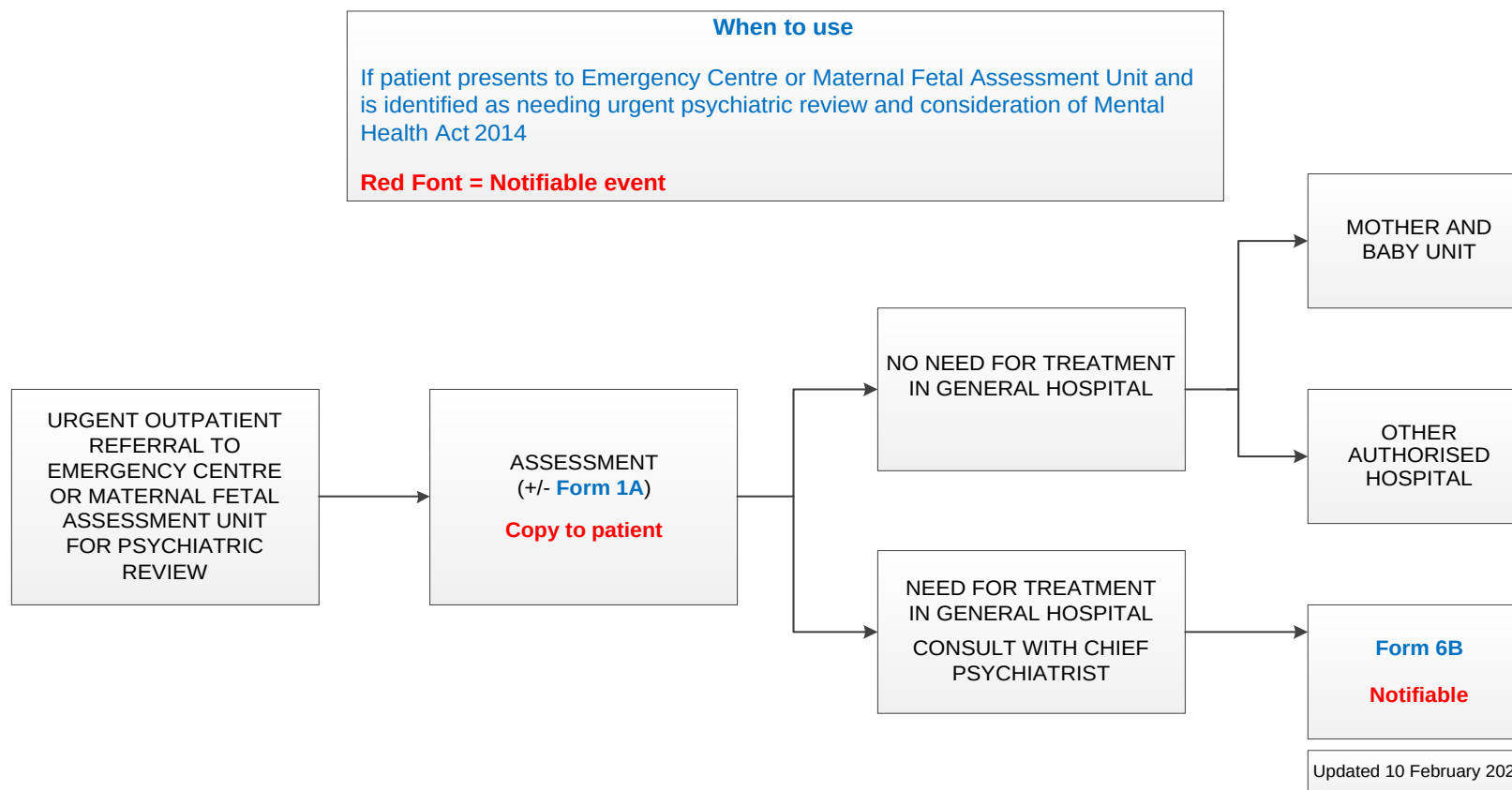
Red Font = Notifiable event





FLOWCHART 4

URGENT OUTPATIENT REFERRAL TO EMERGENCY CENTRE OR MATERNAL FETAL ASSESSMENT UNIT FOR PSYCHIATRIC REVIEW

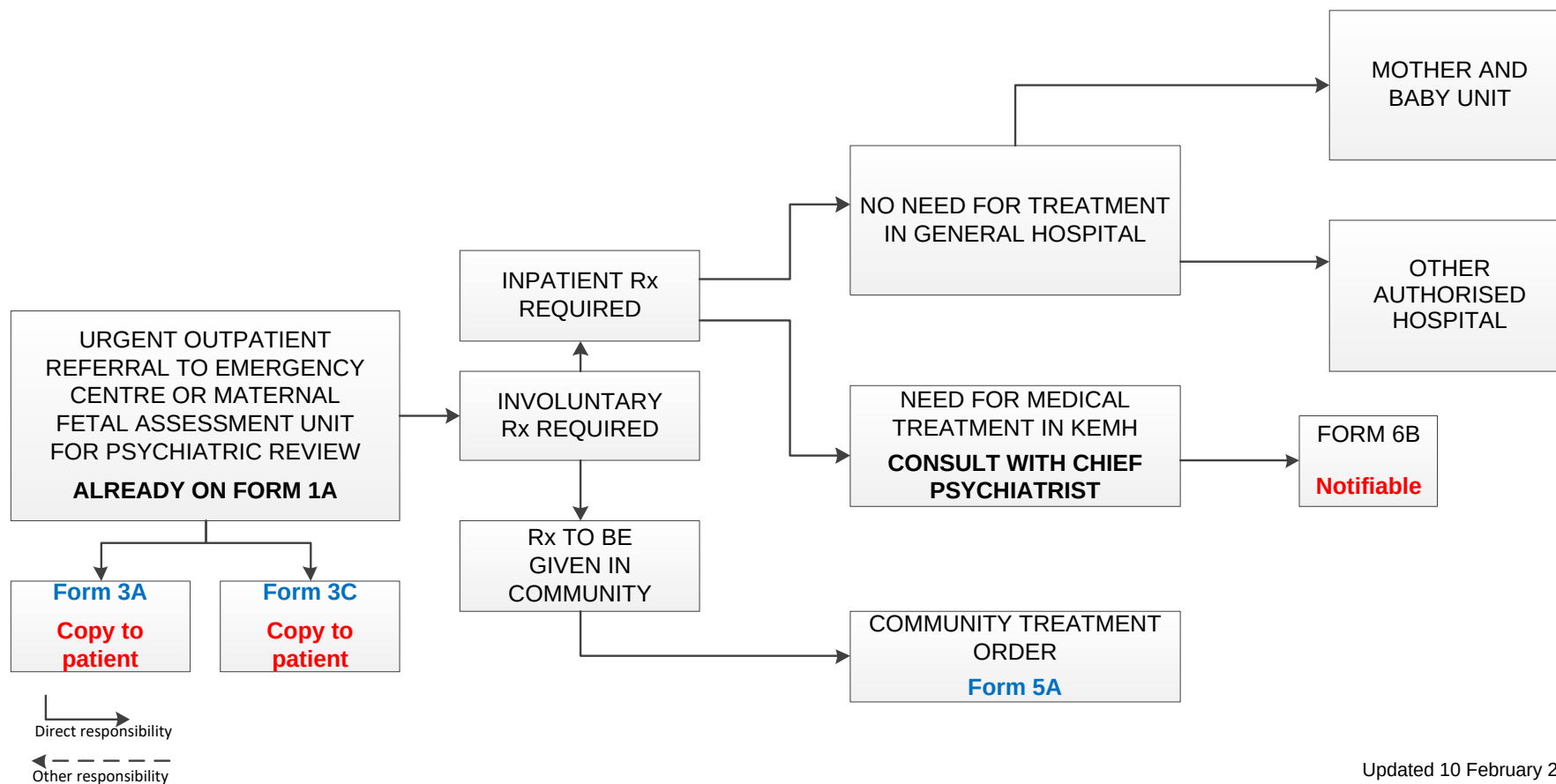


FLOWCHART 5

URGENT OUTPATIENT REFERRAL TO EMERGENCY CENTRE OR MATERNAL FETAL ASSESSMENT UNIT FOR PSYCHIATRIC REVIEW ALREADY ON FORM 1A

When to use

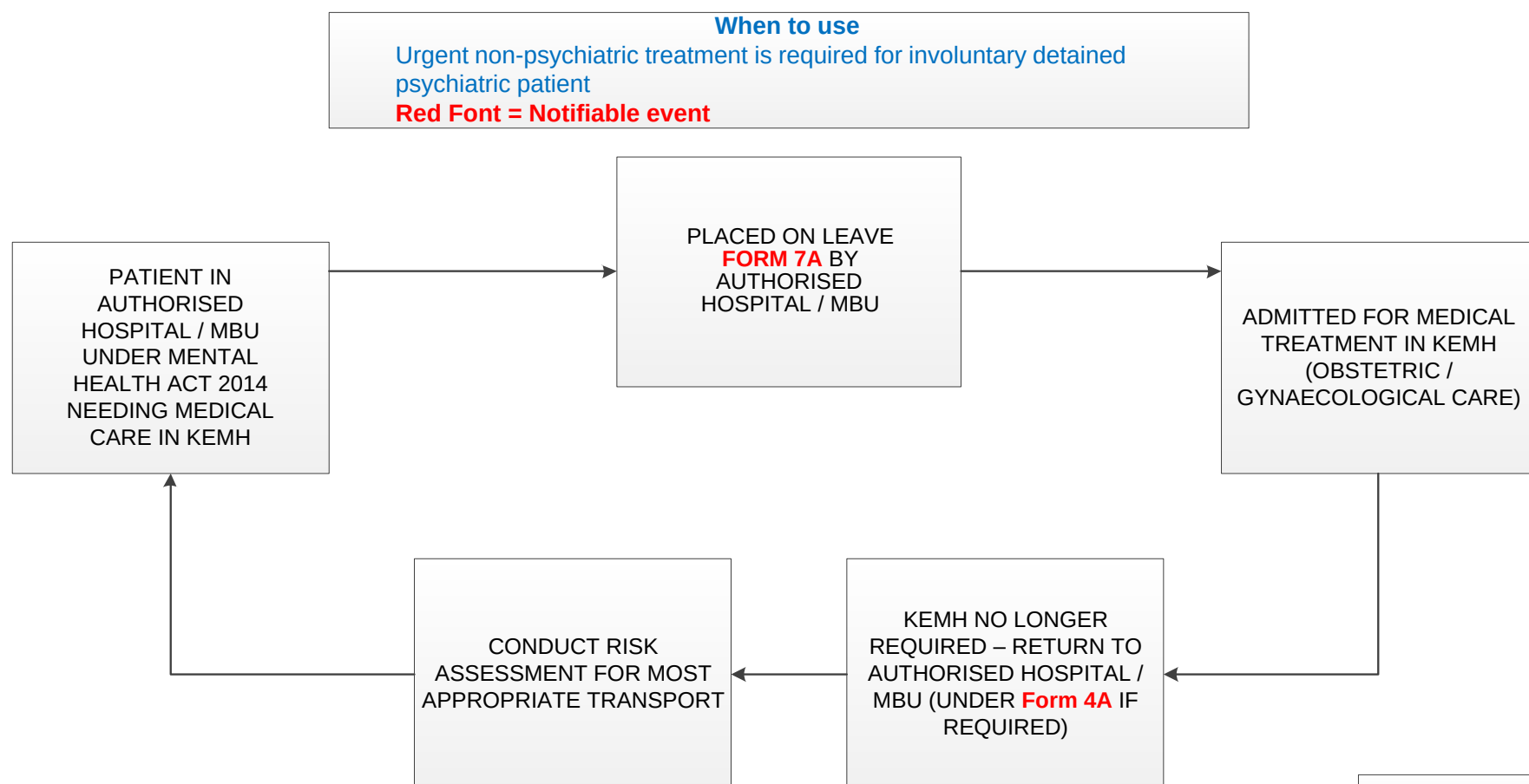
If a patient is referred to Emergency Centre or Maternal Fetal Assessment Unit already on a **Form 1A** needing psychiatric review **Red Font = Notifiable event**





FLOWCHART 6

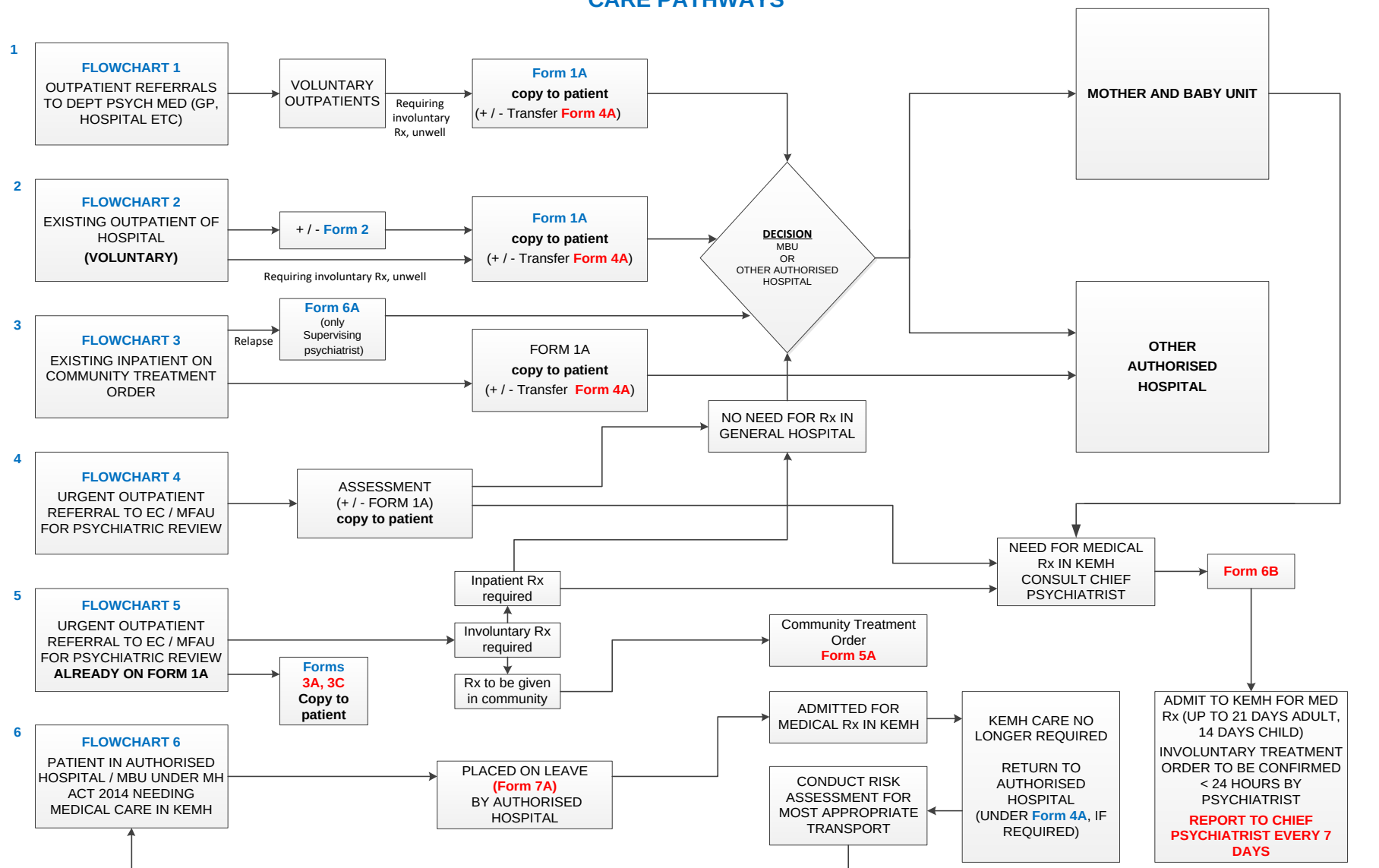
PATIENT IN AUTHORISED HOSPITAL UNDER THE MENTAL HEALTH ACT 2014 NEEDING MEDICAL CARE AT KING EDWARD MEMORIAL HOSPITAL





FLOWCHART FOR WNHS MENTAL HEALTH ACT 2014 CARE PATHWAYS

Red Font = Notifiable event





Government of **Western Australia**
North Metropolitan Health Service
Women and Newborn Health Service



This document can be made available in alternative formats on request.

